Communication Support for Autistic Children

Eunhae Cho | University of Iowa | EALL 7475

My research examines how caregivers support autistic children's communication during family routines and how professional coaching can make that support effective and workable. I focus on children with emerging requesting skills who communicate through speech, gestures, signs, or augmentative and alternative communication (AAC). My long-term goal is to develop a research program connecting intervention effects with families' experiences of using communication support. My preparation in applied linguistics and special education informs this interest in language, learning, and participation. I aim to generate evidence that helps families and professionals choose meaningful goals and interpret progress beyond a single training session.

Research Context and Questions

A functional account of language provides the foundation for this agenda. Skinner (1957) examined verbal behavior in relation to motivational conditions, learning histories, and consequences mediated by listeners. A mand is a request for a currently valued outcome, such as an item, assistance, or continuation of an activity. Its function can be expressed through different forms. My initial work will therefore retain children's existing communication systems and examine independent requests rather than require spoken language. Family routines provide opportunities to study how children's interests, caregivers' responses, and available supports shape these interactions.

Families bring different responsibilities, language practices, and priorities to intervention. A technically accurate procedure may be difficult to use during a busy routine, and an improved implementation score may not capture changes families consider meaningful. Ampuero and

Courtenay's (2026) review supports behavioral skills training for caregiver mand instruction while identifying continued support needs and limited assessment of generalization. This evidence motivates examining child outcomes, caregiver implementation, and family experience together. My contribution will be to connect these sources without assuming that improvement in one establishes improvement in the others.

My initial study focuses on requests for access, assistance, and continuation among autistic children aged three to five. Three questions organize this work:

RQ1. How does the caregiver-training and coaching package affect autistic children's independent requesting during family routines?

RQ2. How does the package affect caregivers' implementation accuracy during those routines?

RQ3. How do caregivers describe their experiences of using and adapting the procedures, and how do these accounts inform the interpretation of outcomes and intervention decisions?

Methodological Approach

I propose a concurrent multiple-probe study across four families, with embedded qualitative description. Following institutional approval, I will recruit caregiver–child dyads through autism services and family organizations. Eligibility will require a documented autism diagnosis, caregiver-identified requesting needs, and observations of absent or prompted requests in at least two routine samples containing observable child interest and time for independent responding. Caregivers will regularly participate in the selected routine, prioritize requesting, and agree to recordings, training, coaching, and interviews. I will serve as coach. One primary routine will support intervention and repeated measurement; a secondary routine will support generalization assessment. Existing communication supports, including AAC, will remain accessible.

Baseline will precede collaborative target planning and instruction. The package will combine communication assessment, collaborative planning, video modeling, behavioral skills training, and ongoing coaching. Initial training will include instruction, modeling, rehearsal, and feedback. Caregivers will practice creating or capturing opportunities, waiting for communication, prompting when needed, and providing the relevant consequence. Weekly coaching will incorporate caregiver reflections and review of recent probes; adjustments and the support provided will be documented. Repeated caregiver errors will prompt focused rehearsal and feedback. Persistently accurate implementation without increased requesting will prompt joint review of child interests, opportunities, and target fit.

To address RQ1, I will measure independent child requests per observable minute. To address RQ2, I will calculate the percentage of applicable caregiver steps completed correctly. Both measures will come from the same ten-minute video-recorded routine probes without live coaching. When probes and coaching occur during one visit, probes will precede coaching; supported practice will be recorded separately. RQ1 and RQ2 will be evaluated through staggered intervention introductions and visual analysis of level, trend, variability, immediacy, overlap, and consistency across families (Kratochwill et al., 2013). Evidence of a functional relation will be evaluated separately for each outcome, requiring at least three convincing demonstrations of change associated with intervention introduction. Secondary-routine probes and follow-up at approximately two and four weeks after scheduled coaching ends will examine generalization and maintenance.

A second observer will independently code at least one third of probes within each family and phase. Coach fidelity will be assessed separately from caregiver accuracy. Scoring definitions will be established before baseline; changes in instructional targets will be

distinguished from changes in measurement rules. Any necessary scoring-rule change will be documented, and scores using different rules will be reported separately. Consent procedures will address recordings and confidentiality. Teaching opportunities will end when children refuse or show continuing distress, and participation will remain voluntary.

RQ3 will draw on intake and exit interviews, weekly reflections, field notes, and decision records. Qualitative description will keep analysis close to caregivers' accounts (Sandelowski, 2000). Coding will examine perceived benefits, feasibility, burden, support needs, and adaptations, while allowing additional categories to emerge. Within-family summaries and cross-family comparisons will trace how experiences and priorities develop. An audit trail, peer review of selected interpretations, and attention to my dual coaching and research roles will support trustworthiness. Caregivers will be invited to clarify summaries of their experiences.

Integration will connect RQ3 with RQ1 and RQ2. Joint displays will align behavioral measures, caregiver accounts, and decisions over time to inform initial planning, intervention reviews, and final interpretation (Fetters et al., 2013). Decision records will distinguish evidence available when each decision was made from retrospective interpretations. Agreement and disagreement will help interpret measured gains alongside families' practical concerns. These analyses will contextualize outcomes without establishing causal mediation.

Research Development and Contributions

During doctoral training, I will use my ongoing review of caregiver-mediated communication research to refine this study and identify gaps in outcome measurement and coaching. My immediate priorities are to finalize procedures, obtain approval, and prepare coding and training materials for data collection. The study will support manuscripts on intervention outcomes and integrated interpretation. I also plan to develop accessible examples

showing how families and practitioners can discuss observed progress alongside implementation concerns.

Subsequent projects will examine how communication modalities, home languages, and family priorities shape goal selection and coaching. Other communicative functions remain important directions for later research. Another direction concerns how special educators learn to collaborate with families and interpret behavioral and experiential evidence. These extensions will require additional partnerships and replication across settings. They will help determine which procedures can be shared across families and which require adaptation. I aim to pursue external funding for this program through partnerships with family organizations and researchers in communication and education.

The agenda connects research with my goals as a teacher educator. Case analyses, observation exercises, and collaborative planning activities can help teacher candidates examine what an intervention asks of both children and caregivers. Partnerships with early childhood and family organizations will support sharing findings and identifying subsequent questions. RQ1 and RQ2 will provide separate evidence about child communication and caregiver implementation; RQ3 will clarify families' experiences and the practical meaning of those outcomes. Conceptually, I aim to clarify how a functional account of communication and family experience can jointly inform judgments about intervention success. These contributions can inform practitioner preparation, the design of family support services, and subsequent studies in community settings.

References

Ampuero, M. E., & Courtenay, S. J. (2026). Using behavioral skills training to teach caregivers to implement mand training with their children with autism spectrum disorders: A systematic review of the single case research. *Journal of Autism and Developmental Disorders.* Advance online publication. https://doi.org/10.1007/s10803-026-07434-w

Fetters, M. D., Curry, L. A., & Creswell, J. W. (2013). Achieving integration in mixed methods designs—Principles and practices. *Health Services Research, 48*(6 Pt 2), 2134–2156. https://doi.org/10.1111/1475-6773.12117

Kratochwill, T. R., Hitchcock, J. H., Horner, R. H., Levin, J. R., Odom, S. L., Rindskopf, D. M., & Shadish, W. R. (2013). Single-case intervention research design standards. *Remedial and Special Education, 34*(1), 26–38. https://doi.org/10.1177/0741932512452794

Sandelowski, M. (2000). Whatever happened to qualitative description? *Research in Nursing & Health, 23*(4), 334–340. https://doi.org/10.1002/1098-240X(200008)23:4%3C334::AID-NUR9%3E3.0.CO;2-G

Skinner, B. F. (1957). *Verbal behavior.* Appleton-Century-Crofts. https://doi.org/10.1037/11256-000